# Pembrolizumab 400 mg + weekly carboplatin AUC 1.5 / paclitaxel → dose-dense AC

NCCP 00861 · Source reconciled; independent oncology, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

## Your practical treatment plan

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 1.5 on Days 1, 8 and 15. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately. After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

STEP 1

### Before the first visit

Blood tests, ECG and cardiac review, medicines review, fertility discussion where relevant and the consent conversation come first. Include the prescribed immune baseline checks.

STEP 2

### Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

### Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

### Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

### Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

### PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

### Visit time and transport

Visits include assessment, pharmacy preparation, premedicines, infusions and observation. The first visit and a change in phase can take longer. Ask about transport and any escort needed after sedation or line insertion.

Sources: NCCP 00861 v2 · official regimen · eviQ · PICC patient information · eviQ · implanted port patient information. Source reconciled; independent oncology, pharmacy and patient/PPI review pending.